Medical Drug Clinical Criteria

Subject:	Ustekinumab Agents		
Document #:	CC-0063	**Publish Date**:	10/27/2026
Status:	Revised	**Last Review Date**:	09/14/2026

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Overview

This document addresses the use of Ustekinumab agents (Stelara, Ustekinumab (unbranded), Selarsdi, Imuldosa, Pyzchiva, Otulfi, Starjemza, Steqeyma, Wezlana and Yesintek), monoclonal antibodies which binds to the p40 protein subunit used by both the interleukin-12 and interleukin-23 (IL-12/23) cytokines disrupting their interaction with receptors and thereby inhibiting the release of proinflammatory cytokines and chemokines. Wezlana, Selarsdi, Otulfi, Pyzchiva, Starjemza, Steqeyma, and Yesintek are designated as interchangeable biosimilars to the reference product Stelara. Imuldosa is designated as biosimilar to the reference product Stelara.

Plaque Psoriasis (otherwise known as psoriasis vulgaris): The American Academy of Dermatology (AAD) and National Psoriasis Foundation (NPF) published joint guidelines on the management and treatment of psoriasis with biologics. The guidelines do not include a treatment algorithm or compare biologics to each other or conventional therapy. The guideline notes that patients with mild-moderate disease may be adequately controlled with topical therapy and/or phototherapy while moderate to severe disease may necessitate treatment with a biologic. Biologics approved for psoriasis were studied in a population with 10% or greater BSA involvement. Moderate to severe disease is defined as involvement in greater than or equal to 3% of body surface area (BSA) or involvement in sensitive areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia). Tumor necrosis factor inhibitor (TNFi) biologics, ustekinumab, IL17 inhibitors, and IL23 inhibitors are all recommended as monotherapy treatment options for adult patients with moderate to severe plaque psoriasis. Combination use of TNFi biologics (etanercept, infliximab, adalimumab) and ustekinumab with apremilast is poorly studied and the AAD has given this practice a grade C recommendation based on limited-quality evidence.

Psoriatic Arthritis: The American College of Rheumatology (ACR) guidelines recommend that initial treatment of patients with active severe PsA or concomitant psoriasis should include a TNFi biologic over an oral small molecule (OSM; including methotrexate, sulfasalazine, cyclosporine, leflunomide, and apremilast). For initial therapy, OSMs are preferred over IL-17 and ustekinumab; and may be considered over TNFi biologics in mild to moderate disease without comorbid conditions or in those who prefer oral therapy. Recommendations involving biologics over OSMs as first line therapy are conditional and based on low quality evidence. Evidence cited includes indirect comparisons of placebo-controlled trials, studies with open-label design, and extrapolation from studies in plaque psoriasis. Furthermore, most pivotal trials for TNFi biologics included a study population that were DMARD experienced. Overall, there is a lack of definitive evidence for the safety and efficacy of biologic drugs over conventional therapy for the initial treatment of most patients with psoriatic arthritis. The ACR guidelines also include recommendations for patients whose disease remains active despite treatment with an OSM. Here, TNFi biologics are recommended over other therapies including IL-17 inhibitors, ustekinumab, tofacitinib, and abatacept. When TNFi biologics are not used, IL-17 inhibitors are preferred over ustekinumab; both of which are preferred over tofacitinib and abatacept. For disease that remains active despite TNFi monotherapy, switching to a different TNFi is recommended over other therapies.

Crohn's Disease: Both the American College of Gastroenterology (ACG) and the American Gastroenterological Association (AGA) Guidelines recommend or suggest TNFi , vedolizumab, ustekinumab, risankizumab, guselkumab, mirikizumab, and upadacitinib for induction and maintenance of remission for patients with moderately to severely active UC. The AGA Guidelines provide recommendations for the positioning of advanced therapies. The AGA suggests using a higher efficacy medication (infliximab, adalimumab, vedolizumab, ustekinumab, risankizumab, mirikizumab, guselkumab) rather than a lower efficacy medication (certolizumab pegol, upadacitinib) in individuals who are naïve to advanced therapies. For individuals who have previously been exposed to one or more advanced

therapies, particularly TNFi, AGA suggests using a higher efficacy medication (adalimumab, risankizumab, guselkumab, upadacitinib) or an intermediate efficacy medication (mirikizumab, ustekinumab) rather than a lower efficacy medication (certolizumab pegol, vedolizumab).

Ulcerative Colitis: Both the American College of Gastroenterology (ACG) and the American Gastroenterological Association (AGA) Guidelines recommend or suggest TNFi, ozanimod, etrasimod, ustekinumab, guselkumab, mirikizumab, risankizumab, tofacitinib, upadacitinib, and vedolizumab for induction and maintenance of remission for patients with moderately to severely active UC. The AGA Guidelines provide recommendations for the positioning of advanced therapies. The AGA suggests using a higher efficacy medication (infliximab, vedolizumab, ozanimod, etrasimod, upadacitinib, risankizumab, guselkumab) or intermediate efficacy medication (golimumab, ustekinumab, tofacitinib, mirikizumab) medication rather than a lower efficacy medication (adalimumab) in individuals who are naïve to advanced therapies. For individuals who have previously been exposed to one or more advanced therapies, particularly TNFi, AGA suggests using a higher efficacy medication (tofacitinib, upadacitinib, ustekinumab) or an intermediate efficacy medication (mirikizumab, risankizumab, guselkumab) rather than a lower efficacy medication (adalimumab, vedolizumab, ozanimod, etrasimod).

Clinical Criteria

When a drug is being reviewed for coverage under a member's medical benefit plan or is otherwise subject to clinical review (including prior authorization), the following criteria will be used to determine whether the drug meets any applicable medical necessity requirements for the intended/prescribed purpose.

Ustekinumab Agents [Stelara, Imuldosa, Otulfi, Pyzchiva, Selarsdi, Starjemza, Steqeyma, Ustekinumab (unbranded), Wezlana, or Yesintek]

Initial requests for Stelara (ustekinumab), Imuldosa (ustekinumab-srlf), Otulfi (ustekinumab-aauz), Pyzchiva (ustekinumab-ttwe), Selarsdi (ustekinumab-aekn), Starjemza (ustekinumab-hmny), Steqeyma (ustekinumab-stba), Ustekinumab (unbranded), Wezlana (ustekinumab-auub), or Yesintek (ustekinumab-kfce) may be approved for the following:

I. Crohn disease (CD) when the following criterion is met:
   A. Individual is 2 years of age or older with moderate to severe CD;

OR

II. Psoriatic arthritis (PsA) when the following criteria are met:
   A. Individual is 6 years of age or older with moderate to severe PsA; **AND**
   B. Individual has had an inadequate response to or is intolerant of conventional therapy [nonbiologic DMARDs (such as methotrexate, sulfasalazine, cyclosporine or leflunomide)]; **OR**
   C. Individual has a contraindication to methotrexate, sulfasalazine, cyclosporine or leflunomide;

OR

III. Plaque psoriasis (Ps) when the following criteria are met:
   A. Individual is 6 years of age or older with chronic moderate to severe (that is, extensive or disabling) plaque Ps with either of the following (AAD 2019):
      1. Plaque Ps involving three percent (3%) body surface area (BSA) or greater; **OR**
      2. Plaque Ps involving less than three percent (3%) BSA involving sensitive areas or areas that significantly impact daily function (such as palms, soles of feet, head/neck, or genitalia); **AND**
   B. Individual has had an inadequate response to or is intolerant of phototherapy or other systemic therapy (such as acitretin, cyclosporine, or methotrexate); **OR**
   C. Individual has a contraindication to phototherapy, acitretin, cyclosporine, and methotrexate;

OR

IV. Ulcerative colitis (UC) when the following criterion is met:
   A. Individual is 2 years of age or older with moderate to severe UC;

OR

V. Immunotherapy-related toxicities when each of the following criteria are met (NCCN 2A):
   A. Individual is undergoing immune checkpoint inhibitor therapy for a cancer diagnosis; **AND**
   B. Individual is experiencing diarrhea or colitis as a result of immune checkpoint inhibitor treatment; **AND**
   C. Symptoms persist despite treatment with steroids and/or biologics (infliximab and/or vedolizumab).

Continuation requests for Stelara (ustekinumab), Imuldosa (ustekinumab-srlf), Otulfi (ustekinumab-aauz), Pyzchiva (ustekinumab-ttwe), Selarsdi (ustekinumab-aekn), Starjemza (ustekinumab-hmny), Steqeyma (ustekinumab-stba), Ustekinumab (unbranded), Wezlana (ustekinumab-auub), or Yesintek (ustekinumab-kfce) may be approved if the following criteria are met:

I. Individual has been receiving and is maintained on a stable dose of Stelara/ Imuldosa/Otulfi/Pyzchiva/Selarsdi/ Starjemza/ Steqeyma/Wezlana/Yesintek; **AND**
II. There is confirmation of clinically significant improvement or stabilization in clinical signs and symptoms of the disease.

Requests for Stelara (ustekinumab), Imuldosa (ustekinumab-srlf), Otulfi (ustekinumab-aauz), Pyzchiva (ustekinumab-ttwe), Selarsdi (ustekinumab-aekn), Starjemza (ustekinumab-hmny), Steqeyma (ustekinumab-stba), Ustekinumab (unbranded), Wezlana (ustekinumab-auub), or Yesintek (ustekinumab-kfce) may not be approved for the following:

I. In combination with oral or topical JAK inhibitors, apremilast, ozanimod, etrasimod, deucravacitinib, or any of the following immunomodulators: Other TNF antagonists, IL-23 inhibitors, IL-17 inhibitors, IL-6 inhibitors, IL-1 inhibitors, vedolizumab, abatacept, rituximab, or natalizumab; **OR**
II. When the above criteria are not met and for all other indications.

Step Therapy

Note: When an ustekinumab agent is deemed approvable based on the clinical criteria above, the benefit plan may have additional criteria requiring the use of a preferred[1] agent or agents.

Ustekinumab Step Therapy

A list of the preferred agents is available here.

Commercial requests for diagnosis of Crohn Disease or Ulcerative Colitis:

Requests for a non-preferred ustekinumab agent may be approved when the following criteria are met:
I. Individual is currently on the requested non-preferred agent; **OR**
II. Individual has had a trial and inadequate response or intolerance to TWO preferred agents. (A trial of multiple products with the same active ingredient counts as a trial of ONE preferred agent);
III. Information is provided for why the individual is unable to use ALL preferred agents (or all remaining preferred agents if individual has tried any preferred agent) due to one of the following:
    A. The individual is subject to a warning or contraindication that appears in the labeling of ALL preferred products and is not included in the labeling of the non-preferred product; **OR**
    B. None of the preferred products have activity against the individual's concomitant clinical condition which is covered by the non-preferred product; **OR**
    C. Remaining preferred agents are TNF antagonists and individual has already been exposed to 1 or more TNF antagonists (i.e. individual is unable to use all preferred agents other than TNF antagonists due to one of the reasons above);

Medicaid

Requests for a non-preferred ustekinumab agent may be approved when the following criterion is met:
I. Individual has had a trial of and has an allergy or severe intolerance to an inactive ingredient in the preferred agent which interferes with the individual's ability to use the product, and the same allergy/severe intolerance is not expected with the non-preferred product (NOTE: A non-preferred agent may not be approved based on convenience factors [such as the convenience of a certain dosage form or device, dosage concentration or regimen]).

[1]Preferred, as used herein, refers to agents that were deemed to be clinically comparable to other agents in the same class or disease category but are preferred based upon clinical evidence and cost effectiveness.

Quantity Limits

Ustekinumab Agents Quantity Limits

Drug	Limit

Stelara 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Stelara 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Stelara 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Stelara 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Ustekinumab 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Ustekinumab 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Ustekinumab 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Ustekinumab 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Imuldosa 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Imuldosa 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Imuldosa 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Pyzchiva 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Pyzchiva (ustekinumab-ttwe) 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Pyzchiva 45 mg/0.5 mL single-use prefilled syringe/autoinjector*†^	1 syringe/autoinjector per 84 days (12 weeks)
Pyzchiva 90 mg/1 mL single-use prefilled syringe/autoinjector#^	1 syringe/autoinjector per 84 days (12 weeks)
Otulfi 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill)

	Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Otulfi 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Otulfi 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Otulfi 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Selarsdi 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (, one time fill)
Selarsdi 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Selarsdi 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Selarsdi 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Starjemza (ustekinumab-hmny) 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials one time fill)
Starjemza (ustekinumab-hmny) 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Starjemza (ustekinumab-hmny) 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Starjemza (ustekinumab-hmny) 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Steqeyma 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Steqeyma (ustekinumab-stba) 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Steqeyma 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Steqeyma 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)
Wezlana 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Wezlana 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Wezlana 45 mg/0.5 mL single-use prefilled syringe/autoinjector*†^	1 syringe/autoinjector per 84 days (12 weeks)

Wezlana 90 mg/1 mL single-use prefilled syringe/autoinjector#^	1 syringe/autoinjector per 84 days (12 weeks)
Yesintek 130 mg/26 mL (5 mg/mL) vial	Body weight 10 kg to 25 kg: 10mg/kg one time fill Body weight more than 25 kg to 55 kg: 2 vials (one time fill) Body weight more than 55kg to 85 kg: 3 vials (one time fill) Body weight more than 85 kg [max limit]: 4 vials (one time fill)
Yesintek 45 mg/0.5 mL vial*^	1 vial per 84 days (12 weeks)
Yesintek 45 mg/0.5 mL single-use prefilled syringe*†^	1 syringe per 84 days (12 weeks)
Yesintek 90 mg/1 mL single-use prefilled syringe#^	1 syringe per 84 days (12 weeks)

Override Criteria

*Initiation of therapy for Plaque Psoriasis (Ps) or Psoriatic Arthritis (PsA) in individuals less than or equal to 100 kg (220 lbs.): May approve 1 (one) additional syringe/autoinjector or vial (45 mg/0.5 mL) in the first 84 days (12 weeks) of treatment.

†Initiation of therapy for PsA in individuals greater than 100 kg (220 lbs.): May approve 1 (one) additional syringe/autoinjector (45 mg/0.5 mL) in the first 84 days (12 weeks) of treatment.

#Initiation of therapy for Ps or concomitant PsA and moderate to severe Ps in individuals greater than 100 kg (220 lbs.): May approve 1 (one) additional syringe/autoinjector (90 mg/1 mL) in the first 84 days (12 weeks) of treatment.

^Maintenance therapy for Crohn Disease (CD) and Ulcerative Colitis (UC): May approve 1 (one) 90 mg syringe/autoinjector or 2 (two) 45 mg vials/syringes/autoinjectors every 8 weeks (56 days).

^For CD or UC, may also approve increased dosing, up to 1 (one) 90 mg syringe/autoinjector or 2 (two) 45 mg vials/syringes/autoinjectors every 4 weeks if the following criteria are met:

I. Individual has been treated with standard maintenance dosing (i.e. every 8 weeks) for *at least* 2 doses or 16 weeks; **AND**
II. The increased dosing is being prescribed by or in consultation with a gastroenterologist; **AND**
III. Individual initially achieved an adequate response to standard maintenance dosing but has subsequently lost response, as determined by the prescriber; **OR**
IV. Individual partially responded but had an inadequate response to standard maintenance dosing as determined by the prescriber; **AND**
V. Symptoms, if present, are not due to active infections or any other gastrointestinal disorder other than the primary disease; **AND**
VI. Requested dosing does not exceed up to 1 (one) 90 mg syringe/autoinjector or 2 (two) 45 mg vials/syringes/autoinjectors every 4 weeks.

Initial approval duration for increased dosing for CD or UC: 16 weeks

^Requests for continued escalated dosing for CD and UC may be approved if the following criteria are met:

I. Requested dosing does not exceed up to 1 (one) 90 mg syringe/autoinjector or 2 (two) 45 mg vials/syringes/autoinjectors every 4 weeks; **AND**
II. Individual has subsequently regained response or achieved adequate response following increased dosing, as shown by improvement in signs and symptoms of the disease (including but not limited to reduction in stool frequency/bloody stools, improvement abdominal pain, or endoscopic response); **AND**
III. Individual is not experiencing unacceptable adverse effects from increased dosing; **AND**
IV. Individual will be assessed regularly for dose de-escalation.

Continued approval duration for increased dosing CD or UC: 1 year

^For CD or UC, Increased dosing may not be approved for the following:

I.	Individual has had no response to ustekinumab at standard maintenance dosing (i.e. every 8 weeks); **OR**
II.	Individual is requesting dose escalation in absence of signs and symptoms of the disease (for example, requesting based on results of therapeutic drug level or anti-drug antibody testing alone).

Coding

The following codes for treatments and procedures applicable to this document are included below for informational purposes. Inclusion or exclusion of a procedure, diagnosis or device code(s) does not constitute or imply member coverage or provider reimbursement policy. Please refer to the member's contract benefits in effect at the time of service to determine coverage or non-coverage of these services as it applies to an individual member.

Ustekinumab agents, intravenous dosage form

HCPCS

J3358	Ustekinumab, for intravenous injection, 1 mg [Stelara intravenous]
Q5098	Injection, ustekinumab-srlf (Imuldosa), biosimilar, 1 mg, [when specified as vial for intravenous use]
Q5099	Injection, ustekinumab-stba (Steqeyma), biosimilar, 1 mg, [when specified as vial for intravenous use]
Q5100	Injection, ustekinumab-kfce (Yesintek), biosimilar, 1 mg, [when specified as vial for intravenous use]
Q5138	Injection, ustekinumab-auub (Wezlana), biosimilar, intravenous, 1 mg
Q5164	Injection, ustekinumab-hmny (Starjemza), biosimilar, 1 mg [when specified as vial for intravenous use]
Q9997	Injection, ustekinumab-ttwe (Pyzchiva), intravenous, 1 mg
Q9998	Injection, ustekinumab-aekn (Selarsdi), biosimilar, 1 mg, [when specified as vial for intravenous use]
Q9999	Injection, ustekinumab-aauz (Otulfi), biosimilar, 1 mg, [when specified as vial for intravenous use]

ICD-10 Diagnosis

K50.00-K50.919	Crohn disease [regional enteritis]
K51.00-K51.919	Ulcerative colitis
K52.1	Toxic gastroenteritis and colitis [immunotherapy-related toxicity]
L40.0-L40.9	Psoriasis
R19.7	Diarrhea, unspecified [immunotherapy-related toxicity]

Ustekinumab agents, subcutaneous dosage form

HCPCS

J3357	Ustekinumab, for subcutaneous injection, 1 mg [Stelara subcutaneous]
Q5098	Injection, ustekinumab-srlf (Imuldosa), biosimilar, 1 mg [when specified as single-use prefilled syringe]
Q5099	Injection, ustekinumab-stba (Steqeyma), biosimilar, 1 mg [when specified as single-use prefilled syringe]
Q5100	Injection, ustekinumab-kfce (Yesintek), biosimilar, 1 mg [when specified as single-use prefilled syringe]
Q5137	Injection, ustekinumab-auub (Wezlana), biosimilar, subcutaneous, 1 mg
Q5164	Injection, ustekinumab-hmny (Starjemza), biosimilar, 1 mg [when specified as single-use prefilled syringe]
Q9996	Injection, ustekinumab-ttwe (Pyzchiva), subcutaneous, 1 mg
Q9998	Injection, ustekinumab-aekn (Selarsdi), biosimilar, 1 mg [when specified as single-use prefilled syringe]
Q9999	Injection, ustekinumab-aauz (Otulfi), biosimilar, 1 mg [when specified as single-use prefilled syringe]

ICD-10 Diagnosis

K50.00-K50.919	Crohn disease [regional enteritis]
K51.00-K51.919	Ulcerative colitis
L40.0-L40.9	Psoriasis

Document History

Revised: 09/14/2026
Document History:

- 09/14/2026 – Select Review: Update age for ulcerative colitis indication. Coding Reviewed: No changes.
- 08/14/2026 – Annual Review: Update immunotherapy related toxicities criteria per NCCN. Coding Reviewed: No changes.
- 05/15/2026 – Select Review: Update age for Crohn's Disease indication. Step therapy table updates. Coding Reviewed: Removed HCPCS NOC C9399 and J3590 for both dosage forms of Starjemza effective 6/30/26 and added Q5164 for both dosage forms of Starjemza effective 7/1/26.
- 02/20/2026 – Select Review: Update may not approve criteria; remove exclusions for infection, tuberculosis testing, posterior reversible encephalopathy syndrome, and combination with phototherapy. Step therapy table updates. Coding Reviewed: No changes.
- 02/01/2025 – Step therapy table updates.
- 11/14/2025 – Annual Review: Remove prerequisite conventional therapy requirements for CD and UC; clarify phototherapy exclusion applies to psoriasis and psoriatic arthritis; update step therapies to include convenience note; update quantity limits with administrative updates and additional products; update escalated dosing continued approval duration to 1 year. Step therapy table updates. Coding Reviewed: No changes.
- 12/01/2025 – Step therapy and step therapy table updates.
- 10/01/2025 – Step therapy table updates.
- 08/15/2025 – Select Review: Add new interchangeable biosimilar Starjemza to clinical criteria, step therapy, and quantity limits; wording and formatting updates. Coding Reviewed: Added HCPCS NOC C9399 and J3590 for Starjemza vial for intravenous use and single-use prefilled syringe.
- 6/10/2025 – Coding update: Separated codes by intravenous and subcutaneous dosage forms. Removed ICD-10-CM K52.1 and R19.7 from J3357, Q5137, Q9996.
- 5/16/2025 – Select Review: Add new unbranded ustekinumab product to clinical criteria and quantity limits; add quantity limits for new dosage forms. Step therapy table updates. Coding Reviewed: Updated description of HCPCS Q9998 effective 7/1/25. Removed HCPCS NOC C9399 for Yesintek and Steqeyma effective 6/30/25. Removed HCPCS NOC J3590 for Imuldosa, Yesintek and Steqeyma effective 6/30/25. Added HCPCS Q5098, Q5099, and Q5100 effective 7/1/25. Consolidated codes L40.0-L40.9 into one range.
- 05/01/2025 – Step therapy drug list and table updates.
- 02/21/2025 – Select Review: Add new biosimilar Steqeyma to clinical criteria, step therapy and quantity limits. Add quantity limit to new Wezlana autoinjector dosage form. Coding Reviewed: Added HCPCS NOC C9399 for Yesintek and Steqeyma. Added Steqeyma to HCPCS NOC J3590. Removed Otulfi from J3590 and added Q9999 for Otulfi effective 4/1/25.
- 12/09/2024 – Select Review: Add new biosimilar Yesintek to clinical criteria, step therapy and quantity limits. Add quantity limit to new Selarsdi dosage form. Add step therapy and step therapy table. Coding reviewed: Added Yesintek to HCPCS NOC J3590.
- 11/15/2024 – Annual Review: Add new biosimilars Imuldosa, Pyzchiva and Otulfi to clinical criteria, step therapy, and quantity limits; wording and formatting updates. Coding Reviewed: Added HCPCS Q9996, Q9997, Q9998, all effective 1/1/25. Added HCPCS NOC J3590 for Imuldosa and Otulfi. Added ICD-10-CM K52.1 and R19.7.
- 05/17/2024 – Select Review: Add new biosimilar Selarsdi to clinical criteria, step therapy, and quantity limits. Coding Reviewed: Added HCPCS Q5137, Q5138.
- 11/17/2023 – Annual Review: Add new interchangeable biosimilar Wezlana to clinical criteria and quantity limits; add immunotherapy-related toxicities indication per NCCN; add etrasimod to combination use exclusion for consistency; update contraindication to prior therapy language for clarity; update Crohn's disease and Ulcerative colitis criteria to separate criteria for IV induction dose or subcutaneous therapy; clarify repeat TB testing requirements; add continuation of use language; wording and formatting updates. Coding Reviewed: No changes.
- 08/18/2023 – Select Review: Clarify may not approve section. Coding Reviewed: No changes.
- 03/08/2023 – Update to quantity limit table/override.
- 11/18/2022 – Annual Review: Update combination exclusion use to include additional agents and specify biologic immunomodulators; include examples of conventional therapy per guidelines; add quantity limit override criteria for increased dosing; wording and formatting updates. Coding Reviewed: No changes.

- 08/19/2022 – Select Review: Update age for psoriatic arthritis based on labeling update; update quantity limit for clarity. Coding Reviewed: No changes.
- 11/19/2021 – Annual Review: Remove prior therapy with biologics to align with other agents; update exclusion list for combination use; update loading dose quantity limit to include weight based limits; clarify tuberculosis testing language; wording and formatting changes for clarity. Coding Reviewed: No changes.
- 11/20/2020 – Annual Review: Add continuation of use section; remove 5-ASA products as examples of conventional therapy for Crohn's disease; add additional examples of combination use for clarity; update tuberculosis testing language. Coding Reviewed: No changes.
- 09/14/2020 – Select Review: Update criteria for expanded psoriasis age indication per label. Coding Reviewed: No changes.
- 11/15/2019 – Annual Review: Add treatment of ulcerative colitis to prior authorization and quantity limit override criteria per FDA label, update definition of moderate psoriasis using BSA based on guidelines; update combination therapy criteria for consistency with other agents; wording and formatting changes. Coding reviewed: Add K51.00-K51.919 for UC.
- 09/23/2019 - Administrative update to add drug specific quantity limit.
- 11/16/2018 – Annual Review: Initial P&T review of Stelara Clinical Guideline. Update clinical criteria to delete "active" disease wording. Update criteria to delete requirement agent is being used "to reduce signs and symptoms, maintain clinical response" etc. Add examples of conventional therapy to approval criteria for clarity. Wording and formatting changes to criteria for consistency. HCPCS and ICD-10 Coding Review: No changes.

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Federal and state laws or requirements, contract language, and Plan utilization management programs or policies may take precedence over the application of this clinical criteria.

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CC-0063 Ustekinumab Agents

Commercial Medical Benefit

Crohn Disease		
Effective Date	**Preferred**	**Non-Preferred**
02/01/2026	Avsola Remicade Unbranded Infliximab (Remicade) Entyvio Selarsdi IV Stelara IV Unbranded Ustekinumab IV (Stelara) Skyrizi IV Tremfya IV	Imuldosa Otulfi Pyzchiva Starjemza Steqeyma Wezlana Yesintek
Ulcerative Colitis		
Effective Date	**Preferred**	**Non-Preferred**
02/01/2026	Avsola Remicade Unbranded Infliximab (Remicade) Entyvio Selarsdi IV Stelara IV Unbranded Ustekinumab IV (Stelara) Skyrizi IV Tremfya IV	Imuldosa Otulfi Pyzchiva Starjemza Steqeyma Wezlana Yesintek

Medicaid Medical Benefit

Effective Date	Preferred Agent	Non-Preferred Agent
01/01/2025: FL Healthy Kids	Selarsdi	Otulfi Pyzchiva Stelara Steqeyma Wezlana Yesintek
01/01/2026: MD, NY, WNY 02/01/2026: NJ 07/01/2026: FL Healthy Kids	Selarsdi	Otulfi Pyzchiva Starjemza Stelara Steqeyma Wezlana Yesintek
02/01/2026: DC, GA	Selarsdi	Imuldosa Otulfi Pyzchiva Starjemza Stelara Steqeyma Wezlana Yesintek